

Fosamprenavir/Ritonavir (1400/200 mg q.d.) / Efavirenz	Ampronavir C _{min} : 136% at steady state	Avoid concomitant use of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets and once-daily Fosamprenavir regimen
Saquinavir HCl/Ritonavir (1000/100mg b.i.d.) / Efavirenz	No clinically relevant interaction was noted	Insufficient data are available for making a dosing recommendation for Saquinavir, with or without Ritonavir, when co-administered with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets. Co-administration with Saquinavir, with or without Ritonavir, is not recommended.
Indinavir (800 mg t.i.d.) / Efavirenz	Indinavir AUC ↓ 31%, C _{trough} 140%	Concomitant use of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets with unboosted Indinavir is not recommended.
Indinavir/Ritonavir (800/100 mg b.i.d.) / Efavirenz	Indinavir AUCsS ↓25%, C _{trough} 150%	Concomitant use of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets with boosted Indinavir is only recommended when it is possible to monitor the plasma concentration of Indinavir
Ritonavir (500 mg b.i.d.) / Efavirenz	Interaction studies have shown moderate increases in the AUC for both Ritonavir and Efavirenz.	Avoid concomitant use of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets with full-dose Ritonavir, due to low tolerability.
Nelfinavir (various doses) / Efavirenz	Interaction studies have shown variable results, including a 20% increase in Nelfinavir AUC and C _{min} , as well as a 25% decrease in AUC and 45% decrease in C _{min} .	Concomitant use with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets is only recommended when it is possible to monitor the plasma concentration of Nelfinavir.
Lopinavir/Ritonavir soft capsules or oral solution / Efavirenz Lopinavir/Ritonavir tablets (400/100 mg b.i.d.) (500/125 mg b.i.d.) / Efavirenz Lopinavir/Ritonavir (400 mg/100 mg b.i.d.) /Tenofovir	Substantial decrease in Lopinavir exposure Lopinavir C _{min} ↓≈ 40% Lopinavir/Ritonavir 400/100 mg twice daily without Efavirenz Lopinavir/Ritonavir: No significant effect on Lopinavir/Ritonavir PK parameters. Tenofovir: AUC: ↑ 32% C _{max} : ↔ C _{min} : ↑ 51%	Insufficient data are available to make a dosing recommendation for Lopinavir/Ritonavir when dosed with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets Co-administration of Lopinavir/Ritonavir and Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets is not recommended.
Atazanavir/Ritonavir /Tenofovir Disoproxil Fumarate (300 mg q.d./100 mg q.d./300 mg q.d.)	Atazanavir: AUC: ↓ 26% (↓ 42 to ↓ 3) C _{max} : ↓ 28% (↓ 50 to ↑ 5) C _{min} : ↓ 26% (↓ 46 to ↑ 10) Co-administration of Atazanavir/Ritonavir with Tenofovir resulted in increased exposure to Tenofovir. Higher Tenofovir concentrations could potentiate Tenofovir-associated adverse events, including renal disorders.	Co-administration of Atazanavir/Ritonavir and Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets is not recommended.
Atazanavir/Ritonavir/ Efavirenz (400 mg q.d./100 mg q.d./600 mg q.d., all administered with food) Atazanavir/Ritonavir/ Efavirenz (400 mg q.d./200 mg q.d./600 mg q.d., all administered with food)	Atazanavir: AUC: ↔** (↓ 9% to ↑ 10%) C _{max} : ↑ 17%* (↑ 8 to ↑ 27) C _{min} : ↓ 42%* (↓ 31 to ↓ 51) Atazanavir: AUC: ↔** (↓ 10% to ↑ 26%) C _{max} : ↔** (↓ 5% to ↑ 26%) C _{min} : ↑ 12%*** (↓ 16 to ↑ 49) (CYP3A4 induction). * When compared to Atazanavir 300 mg/ Ritonavir 100 mg q.d. in the evening without Efavirenz. This decrease in Atazanavir C _{min} might negatively impact the efficacy of Atazanavir. ** based on historical comparison. Co-administration of Efavirenz with Atazanavir/Ritonavir is not recommended.	The clinical significance of the changes in Darunavir and Efavirenz concentrations has not been established, and may vary depending on, e.g., whether there is clinically significant resistance to Darunavir. Darunavir/Ritonavir should be used with caution in combination with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets.
Tipranavir/Ritonavir / Efavirenz	Appropriate data on the interaction between the approved Tipranavir regimen and Efavirenz are lacking.	The combination of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets and Tipranavir/Ritonavir should be avoided.
Darunavir/Ritonavir (300/100 mg b.i.d.) / Efavirenz Darunavir/Ritonavir (300 mg/100 mg b.i.d.) / Tenofovir	Darunavir AUC at steady state ↓13%, C _{min} ↓ 31%, Efavirenz AUC ↑ 21%, C _{min} ↑ 17% Darunavir: No significant effect on Darunavir/ Ritonavir PK parameters. Tenofovir: AUC: ↑ 22%, C _{min} : ↑ 37%	The clinical significance of the changes in Darunavir and Efavirenz concentrations has not been established, and may vary depending on, e.g., whether there is clinically significant resistance to Darunavir. Darunavir/Ritonavir should be used with caution in combination with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets.
CCR-5 antagonists		
Maraviroc (100 mg b.i.d.) / Efavirenz 600 mg q.d	Maraviroc AUC: ↓ 45% Maraviroc C _{max} : ↓ 51%	When co-treating with maraviroc and Efavirenz in the absence of a boosted PI, the maraviroc dose should be increased to 600 mg twice daily. For other combinations, please refer to the SmPC for the medicinal product containing maraviroc.
Integrase strand transfer inhibitors		
Raltegravir (400 mg single dose) / Efavirenz Raltegravir (400 mg b.i.d.) / Tenofovir	Raltegravir AUC ↓ 36% Raltegravir AUC ↑ 49% Raltegravir C _{max} ↑ 64%	No dosage adjustment is necessary if Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets and Raltegravir are co-administered.
Antifungals		
Ketoconazole (400 mg single dose, Efavirenz 600 mg to steady state) / Efavirenz	Ketoconazole AUC ↓ 72%	Consider alternative antifungal agent, or use therapeutic drug monitoring (TDM) if available.
Itraconazole (200 mg b.i.d.) / Efavirenz	Itraconazole AUC at steady state ↓ 39%, C _{min} ↓ 44%	Consider alternative antifungal agent, or use TDM if available
Posaconazole (400 mg b.i.d./400 mg q.d.) / Efavirenz	Posaconazole: AUC 150% C _{max} ↓ 45%	Concomitant use of Posaconazole and Efavirenz should be avoided.
Fluconazole (200 mg q.d.) / Efavirenz	No significant interaction	
Voriconazole (200 b.i.d.) / Efavirenz (600mg)	No data available	Efavirenz and voriconazole at standard doses must not be co-administered.
Voriconazole (200 mg b.i.d.) / Efavirenz 400 mg q.d)	Voriconazole AUCsS: ↓77%; Efavirenz AUCsS: 44%	The dose reduction for Efavirenz with voriconazole at standard dose leads to a significant alteration in the pharmacokinetics of both drugs and must thus not be used.
Voriconazole (400 mg b.i.d.) / Efavirenz 300 mg q.d	Voriconazole AUCsS: 17%; Efavirenz AUCsS: 117%; both compared with standard doses of voriconazole and Efavirenz (200 mg b.i.d and 600 mg q.d, respectively)	If coadministration is considered necessary, voriconazole should be dosed 400 mg b.i.d and Efavirenz dosed at 300 mg q.d. As this dose reduction of Efavirenz cannot be accommodated for with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets, alternative formulations of Efavirenz, Tenofovir and Lamivudine should be used.
Antibacterials/Antituberculotics		
Clarithromycin (500 mg b.i.d, multiple doses) / Efavirenz	Clarithromycin AUC ↓ 39%; 14-OH-clarithromycin AUC ↑ 34%	The clinical significance, if any, of these alterations in Clarithromycin exposure are not known. A high frequency of rash was seen when the drugs were co-administered in healthy volunteers. Consider Azithromycin instead, if possible
Azithromycin (600 mg single dose) / Efavirenz (400 mg once daily),	No clinically significant pharmacokinetic interaction	No dosage adjustment is necessary for either medicinal product
Rifampicin (600 mg q.d, multiple doses)/ Efavirenz	Efavirenz AUC 126%, C _{min} 132%	When co-treating, a dose increase of Efavirenz from 600mg to 800 mg q.d. should be considered.
Rifabutin (300 mg q.d.) / Efavirenz	Rifabutin AUCsS 138%	Increase Rifabutin dose by 50% if co-treating with
Antimalarials		
Atovaquone Chloroquine Mefloquine Proguanil , Sulfadoxine Pyrimethamine / Efavirenz	No formal interaction studies available. Drug interactions and safety in coadministration with Efavirenz has not been systematically evaluated; on a theoretical basis, clinically significant drug interactions with Efavirenz are unlikely	

Amodiaquine/artesunate (600/250 mg q.d.) / Efavirenz	An interaction study (EFV at steady-state) was terminated after the first two subjects developed asymptomatic but significant hepatic enzyme elevations after a three-day course of Amodiaquine. Amodiaquine AUC ↑ 114 and 302% respectively	Possibly increased hepatic toxicity. Avoid combination.
Quinine / Efavirenz	No formal interaction study available Quinine is extensively metabolised by CYP3A. Coadministration with Efavirenz may decrease quinine exposure, and reduce the antimalarial effect.	If possible, an alternative agent to quinine should be used in co-treatment with Efavirenz
Lumetantrine, Halofantrine / Efavirenz	No formal interaction studies available These agents are metabolised by CYP3A; hence, co-treatment with Efavirenz may decrease exposure.	Co-treatment is not recommended.
Artemisinin and its derivatives / Efavirenz	No formal interaction studies available Artemisinin and its derivatives are transformed into active metabolites by CYP3A. Exposure may be decreased by Efavirenz. Empirical data are lacking and possible clinical consequences are unknown	
ANTIVIRALS AGAINST HBV		
Adefovir Dipivoxil / Tenofovir	AUC: ↔ C _{max} : ↔	Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets should not be administered concurrently with Adefovir Dipivoxil due to an expected lack of additive effect
Entecavir (1 mg q.d.)	Carbamazepine AUCsS: ↓27%, C _{min} 155%; Efavirenz AUCsS: ↓36%, C _{min} 147%	Co-administration should be avoided unless plasma concentrations of Carbamazepine and Efavirenz can be monitored
Phenytoin / Efavirenz	No interaction study available. Phenytoin and Efavirenz clearance is likely to be increased.	Co-administration should be avoided unless plasma concentrations of Carbamazepine and Efavirenz can be monitored.
Valproic acid (250 mg b.i.d.) / Efavirenz	No significant interaction is likely.	
Vigabatrin	No significant interaction is likely	Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets and Vigabatrin can be co-administered without dose adjustment.
CARDIOVASCULAR AGENTS		
Calcium channel blockers		
Diltiazem (240 mg q.d.) / Efavirenz	Diltiazem: AUC: ↓ 69% Desacetyl diltiazem: AUC: 175% N-monodemethyl diltiazem: AUC: 137%	Monitor the clinical effect of diltiazem and increase dose if necessary.
Verapamil, Felodipine, Nifedipine, Nicardipine / Efavirenz	Interaction not studied. Calcium channel blocker exposure is likely to be lowered in co-treatment with Efavirenz.	Monitor clinical effect and increase calcium channel blocker dose if necessary.
LIPID LOWERING AGENTS		
Atorvastatin (10 mg q.d.) / Efavirenz	Atorvastatin: AUC: 143% Total active moiety: AUC: ↓ 34%	Cholesterol levels should be periodically monitored and the dose of Atorvastatin increased in case of insufficient efficacy
Pravastatin (40 mg q.d.) / Efavirenz	Pravastatin: AUC: ↓ 40%	Cholesterol levels should be periodically monitored and the dose of pravastatin increased in case of insufficient efficacy
Simvastatin 40 mg q.d.) / Efavirenz	Simvastatin: AUC: 169% Total active moiety: AUC: ↓ 60%	Cholesterol levels should be periodically monitored and the dose of simvastatin increased in case of insufficient efficacy.
Rosuvastatin / Efavirenz	Interaction not studied. Rosuvastatin is largely excreted unchanged via the faeces; therefore metabolic drug interaction with Efavirenz is not expected	
HORMONAL CONTRACEPTIVES		
Ethinylestradiol/Norgestimate (0.035 mg + 0.25 mg q.d.) / Efavirenz	No change in Ethinylestradiol exposure. Levonorgestrel AUC ↓ 83%, Norelgestromin AUC ↓ 64% (active metabolites).	A reliable method of barrier contraception should be used in addition to oral contraceptives
DMPA (150 mg i.m. single dose) / Efavirenz	The pharmacokinetics and efficacy of DMPA was not altered due to co-treatment with Efavirenz	Because of the limited information available, a reliable method of barrier contraception must be used in addition to hormonal contraception
Etonogestrel (implant) / Efavirenz	Interaction not studied. Decreased exposure of Etonogestrel may be expected due to the CYP3A induction of Efavirenz. There have been occasional postmarketing reports of contraceptive failure with Etonogestrel in Efavirenz-exposed patients	A reliable method of barrier contraception must be used in addition to hormonal contraception.
IMMUNOSUPPRESSANTS		
Tacrolimus, Cyclosporine, Sirolimus / Efavirenz	Interaction not formally studied. Decreased exposure of these immunosuppressants may be expected when co-treating with Efavirenz.	Dose adjustments of the immunosuppressants may be needed. Close monitoring of immunosuppressant drug concentrations for at least 2 weeks (until steady-state concentrations are reached) is recommended when starting or stopping therapy with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets.
OTHERS		
Methadone / Efavirenz	Methadone AUC ↓ 52%	Monitor for withdrawal symptoms and increase methadone dose if necessary.
Buprenorphine / Efavirenz	Buprenorphine AUC ↓ 50%; Norbuprenorphine AUC ↓ 71% (active metabolite) Despite these decreases in exposure, no patients in the study exhibited withdrawal symptoms	Monitor for withdrawal symptoms and increase Buprenorphine dose if necessary.
Warfarin / Efavirenz	No interaction study available Co-administration may decrease (and less likely increase Warfarin exposure.	Monitor INR. Dose adjustments of Warfarin may be necessary.
Lorazepam (2mg single dose) / Efavirenz	Lorazepam: AUC: ↑ 7% (↑ 1 to ↑ 14)	No dose adjustment necessary
Midazolam, Triazolam / Efavirenz	No interaction study available	These benzodiazepines are metabolised by CYP3A. While Efavirenz is an inducer of CYP3A in vivo, it acts as an inhibitor in vitro. The impact of coadministration on Midazolam and triazolam pharmacokinetics is unknown. Co-administer with caution
St. John's Wort (<i>hypericum perforatum</i>) / Efavirenz	No interaction study available	Concomitant treatment contraindicated. Co-administration likely to decrease Efavirenz levels and to precipitate virological failure.

Pregnancy and Lactation
Women of childbearing potential:
Based on the animal data, it is recommended that pregnancy should be avoided in women treated with Efavirenz, one of the components of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets. Barrier contraception should always be used in combination with other methods of contraception (for example, oral or other hormonal contraceptives). Because of the long half-life of Efavirenz, use of adequate contraceptive measures for 12 weeks after discontinuation of Efavirenz is recommended. Women of childbearing potential should undergo pregnancy testing before initiation of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets.
Pregnancy:
Studies of Efavirenz in animals have shown reproductive toxicity, including marked teratogenic effects. Cases of neural tube defects in infants born to women with first trimester exposure have been reported. Animal studies do not indicate direct or indirect harmful effects of Tenofovir Disoproxil Fumarate with respect to pregnancy, foetal development, parturition or postnatal development. In humans, the safety of Tenofovir in pregnancy has not been fully established. Sufficient numbers of first trimester exposures have been monitored, however, to detect at least a twofold increase in the risk of overall birth defects. No increase in birth defects was seen.
No increased risk of birth defects has been reported for Lamivudine. However, risks to the fetus cannot be ruled out.
Due to the possible teratogenic effects of Efavirenz, Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets should not be used during the first trimester of pregnancy, and only used during the subsequent trimester if the benefit is considered to outweigh the risk.
Breast-feeding:
In animal studies it has been shown that Tenofovir is excreted into milk. It is not known whether Tenofovir is excreted in human milk. Lamivudine is excreted into the breast milk of lactating mothers. It is not known whether Efavirenz is excreted in human milk. Current recommendations on HIV and breastfeeding (e.g. those from the WHO) should be consulted before advising patients on this matter. Preferred options may vary depending on the local circumstances
Side Effects
The following adverse events have been reported in controlled clinical trials during treatment of HIV-1 infection with Efavirenz, Lamivudine

and/or Tenofovir Disoproxil Fumarate.
The adverse events considered at least possibly related to the treatment are listed below by body system, organ class and absolute frequency. Frequencies are defined as very common (≥ 1/10), common (≥ 1/100, <1/10), uncommon (≥1/1000, <1/100), rare (≥1/10,000, <1/1000), very rare (<1/10,000). In addition, adverse events identified during post-approval use are listed (frequency category: "not known"). Since they are reported voluntarily from a population of unknown size, estimates of frequency cannot be made. These events have been included for their potential causal connection to the active components of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets, taking also into account their seriousness and the number of reports.
Metabolic and nutrition disorders
Very common: increases in fasting triglycerides, total cholesterol, high- and low-density lipoprotein cholesterol, hypophosphataemia
Rare: lactic acidosis
Not known: lipodystrophy, hypokalaemia
Blood and lymphatic systems disorders
Uncommon: neutropenia, anaemia, thrombocytopenia
Very rare: pure red cell aplasia
Respiratory, thoracic and mediastinal disorders
Common: cough, nasal symptoms
Very rare: dyspnoea
Nervous system disorders
Very common: dizziness
Common: abnormal dreams, disturbance in attention, headache, insomnia, somnolence
Uncommon: agitation, amnesia, ataxia, abnormal coordination, confusional state, convulsions, abnormal thinking
Very rare: peripheral neuropathy (paresthesiae)
Not known: tremor
Psychiatric disorders
Common: anxiety and depression
Uncommon: affect ability, aggression, euphoric mood, hallucination, mania, paranoia, suicide attempt, suicidal ideation
Not known: neurosis, completed suicide
Hepatobiliary disorders
Common: elevation of liver enzymes
Uncommon: acute hepatitis
Not known: hepatic failure, hepatic steatosis
Renal and urinary disorders:
Rare: acute renal failure, renal failure, proximal renal tubulopathy (including Fanconi syndrome), increased serum creatinine
Very rare: acute tubular necrosis
Unknown: nephritis (including acute interstitial nephritis), nephrogenic diabetes insipidus
Skin and subcutaneous tissue disorders
Very common: rash
Common: pruritus, hair loss
Uncommon: erythema multiforme, Stevens-Johnson syndrome
Not known: photo allergic dermatitis
Musculoskeletal and connective tissue disorders
Common: arthralgia, myalgia
Not known: rhabdomyolysis, osteomalacia (manifested as bone pain and infrequently contributing to fractures), muscular weakness, myopathy, osteonecrosis (see section 4.4.)
Reproductive system and breast disorders
Uncommon: gynaecomastia
Eye disorders
Uncommon: blurred vision
Ear and labyrinth disorders
Uncommon: vertigo
Not known: tinnitus
Gastrointestinal disorders
Very common: diarrhoea, nausea, vomiting
Common: abdominal pain, flatulence
Uncommon: acute pancreatitis
General disorders and administration site disorders
Common: fatigue, malaise, fever
Not known: immune reconstitution syndrome (see section 4.4), flushing
Description of selected adverse reactions
Renal tubulopathy

The following adverse reactions, listed under the body system headings above, may occur as a consequence of proximal renal tubulopathy due to Tenofovir Disoproxil Fumarate: rhabdomyolysis, osteomalacia (manifested as bone pain and infrequently contributing to fractures), hypokalaemia, muscular weakness, myopathy and hypophosphataemia. These events are not considered to be causally associated with Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets therapy in the absence of proximal renal tubulopathy.
Nervous system symptoms
Nervous system symptoms are common with Efavirenz, one of the components of Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets. In clinical controlled studies of Efavirenz, nervous system symptoms of moderate to severe intensity were experienced by 19% (severe 2%) of patients, and 2% of patients discontinued therapy due to such symptoms. They usually begin during the first one or two days of Efavirenz therapy and generally resolve after the first two to four weeks. They may occur more frequently when Efavirenz 600 mg, Lamivudine 300 mg and Tenofovir Disoproxil Fumarate 300 mg Tablets is taken concomitantly with meals possibly due to increased Efavirenz plasma levels (see section 5.2). Dosing at bedtime seems to improve the tolerability of these symptoms (see section 4.2).
Bone effects of Tenofovir in adolescents
The effect of Tenofovir on bone mass in those not fully grown is a specific theoretical safety concern. Assessment of adverse reactions is based on one randomized trial in 87 HIV-1 infected paediatric subjects (12 to <18 years of age) who received treatment with Tenofovir (N=45) or placebo (N=42) in combination with other antiretroviral agents for 48 weeks. Bone effects observed in paediatric subjects 12 years of age and older, such as increased bone turnover were consistent with those observed in adult clinical trials (see section 4.4).
Symptoms and Treatment of Overdose
If overdose occurs the patient must be monitored for evidence of toxicity, and standard supportive treatment applied as necessary. Some patients accidentally taking Efavirenz 600 mg twice daily have reported increased nervous system symptoms. One patient experienced involuntary muscle contractions. Administration of activated charcoal may be used to aid removal of unabsorbed Efavirenz. There is no specific antidote for overdose with Efavirenz. Since Efavirenz is highly protein bound, dialysis is unlikely to remove significant quantities of it from blood.

Tenofovir can be removed by haemodialysis; the median haemodialysis clearance of Tenofovir is 134 ml/min. The elimination of Tenofovir by peritoneal dialysis has not been studied.

Because a negligible amount of Lamivudine was removed via (4-hour) haemodialysis, continuous ambulatory peritoneal dialysis, and automated peritoneal dialysis, it is not known if continuous haemodialysis would provide clinical benefit in a Lamivudine overdose event

List of excipients
Tablet core:
Croscarmellose Sodium, Microcrystalline Cellulose, Hydroxypropylcellulose, Magnesium Stearate, Sodium Lauryl Sulfate, Ferric Oxide, Hypromellose
Film-coating:
Poly Vinyl Alcohol, Macrogol 3350, Titanium dioxide, Talc
Storage
Do not store above 30°C.
Shelf Life
≥6 months.
Presentations
30 Tablets, 90 Tablets and 100 Tablets are packed in HDPE Container and closed with closure containing induction sealing wad are packed in printed carton along with pack insert.
Manufactured by:
 **AUROBINDO**
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Unit VII, SEZ, TSIC,
Plot. No. 51, S.No.5, H11/P, 425/P, 434/P, 435/P & 458/P,
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